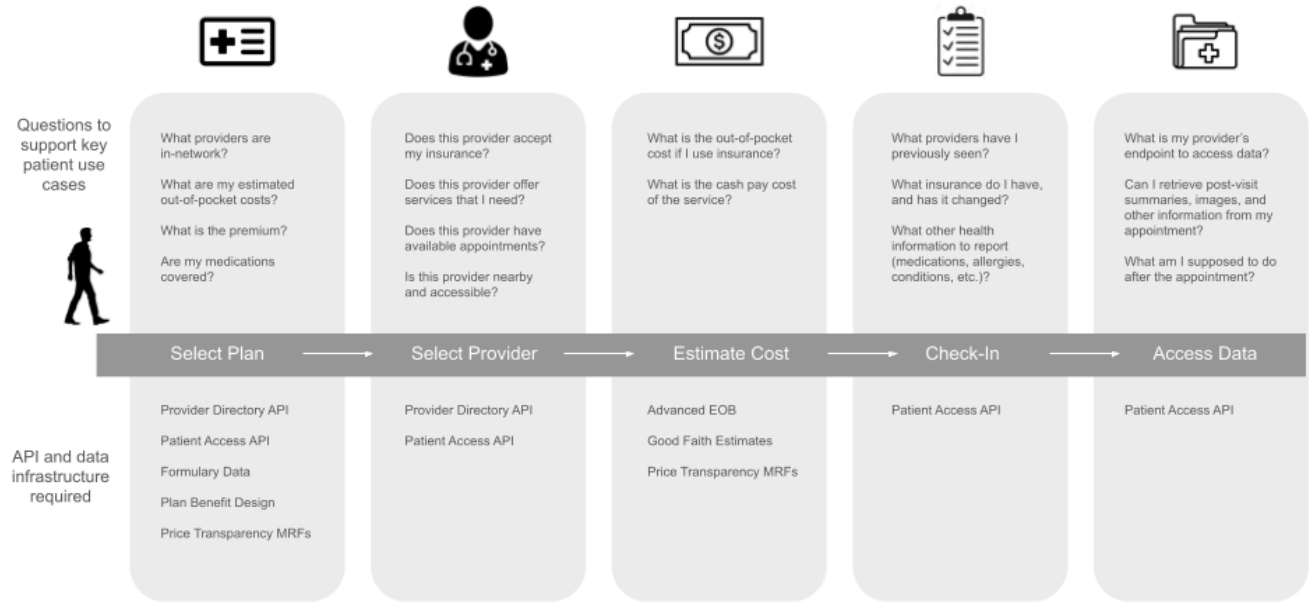
Defacto Health Response to Health Technology Ecosystem RFI
Centers for Medicare & Medicaid Services CMS-0042-NC

We appreciate the opportunity to respond to the CMS Request for Information (RFI) on the Health Technology Ecosystem. This response is informed by Defacto Health's expertise in provider directories and interoperability. We hope the information provided will be useful to CMS as it considers how to lead infrastructure progress to empower Medicare beneficiaries and improve the state of data interoperability.

As requested, Defacto Health has addressed select questions included in the RFI and included the question labels, text of the questions, and the answers together for your convenience. Defacto's responses revolve around 4 use cases:

1) Individual selecting a plan
2) Individual selecting a provider
3) Individual estimating cost of service
4) Individual checking into an appointment and accessing their data

They are described in more detail in our article '[Empowering Patient Choice with Payer and Provider Data]' which was published in January 2025.



Patient Needs

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PC-1. What health management or care navigation apps would help you understand and manage your (or loved ones) health needs, as well as the actions you should take?

As an individual and a caregiver, apps that would be most impactful support key decisions in my and my loved ones' healthcare journeys: choosing the right insurance plan, selecting the right provider, understanding my cost of care, checking into appointments, and accessing health data. These are universal moments for all individuals interacting with the healthcare system. Making these moments easier would improve the experience for Medicare beneficiaries seeking care and coverage.

These are also foundational or "apex" use cases. If they are supported by robust interoperability and data infrastructure, they can enable a range of more specific applications. Without reliable data and interoperability, even well-designed apps cannot address these use cases for beneficiaries. The success of these use cases is a proxy for the overall maturity of the health tech ecosystem. They support key decisions by patients, providers, and plans that promote high quality, sustainable healthcare networks and can lead to a more dynamic healthcare system.

a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

1. *Find the right in-network provider - a provider who is nearby, high-quality, and with available appointments. With average wait times of 38 days (and up to 70 days in some metro areas), and 1 in 5 appointment requests leading to dead ends due to unknown availability, patients face serious barriers. Inaccurate payer directories, estimated to be 50% inaccurate on average, make this even harder.*[1]

2. *Select the right health plan - one that includes preferred providers, is affordable, and covers the needed services and medications. 40% of people report not having enough information to choose the right plan for their needs.*[2]

3. *Understand out-of-pocket costs - for services across in-network providers. While half of U.S. adults say healthcare is difficult to afford, 73% of consumers don't know they can look up prices, and only 20% have done so before a visit.*[3]

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[1] https://www.ecgmc.com/insights/whitepaper/the-waiting-game-new-patient-appointment-access-for-us-physicians

[2] https://www.kff.org/private-insurance/issue-brief/navigating-the-maze-a-look-at-health-insurance-complexities-and-consumer-protections/

[3] https://pmc.ncbi.nlm.nih.gov/articles/PMC11671781/

Digital tools that help solve these problems would significantly improve the healthcare experience for individuals and families. It is essential that the necessary infrastructure and data is available to support these use cases.

b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

1. *Find the first available in-network provider and book an appointment – I want the assistant to check across my health plan's network directory, filter for providers with timely availability, filter for providers in the appropriate specialty (that matches the type of care I wish to receive), filter for providers who are nearby and accessible, confirm that they're accepting new patients, and book the earliest available appointment, syncing with my calendar and notifying me with check-in instructions. Today, this process often involves multiple phone calls and web searches, many of which lead to outdated or incorrect information.*

2. *Find the best health plan for me – The assistant should be able to review available health plans based on my existing conditions and medications, ensure my preferred providers and facilities are in-network, and simulate total costs (premiums + expected out-of-pocket). It would be especially helpful if it could highlight trade-offs. For example, a lower premium plan that excludes my primary care doctor versus a higher premium plan that includes my PCP.*

3. *Help me understand and compare out-of-pocket costs – I'd ask the assistant to give me an estimate of what I'll pay for a service, like an MRI or lab test, based on my deductible, copay, and the negotiated rates with nearby in-network providers. It would be even more useful if it could surface relative cost and quality scores of the provider, so I can weigh options like convenience vs. cost.*

PC-4. What features are missing from apps you use or that you are aware of today?

a. What apps should exist but do not yet? Why do you believe they do not exist yet?

Apps that support plan shopping, provider search, and cost estimation exist today, but their utility is limited because the underlying data is outdated, incomplete, or inaccurate. The core issue is lack of data integrity upstream from payers and providers. Without accurate, timely, and standardized data, these apps cannot do their intended job. The incentives for data holders to make this information accessible and interoperable are weak, which hinders the development of truly useful digital tools for consumers. In

addition, while there are mandates that require that this data be made available, they are either unenforced, or lack specific requirements to make the data usable.

b. What set of workflows do you believe CMS is uniquely positioned to offer?

CMS is uniquely positioned to ensure the availability, accuracy, and readiness of core data infrastructure, including Provider Directory APIs, Price transparency machine-readable files, Patient Access APIs, and other mandated APIs and MRFs.

Rather than building full-featured consumer apps, CMS should focus on certifying these APIs, monitoring data quality, and enforcing compliance. While CMS may choose to pilot tools like a plan shopping app or universal provider directory, its greater impact lies in making trusted, standardized data publicly accessible, and allowing third-party developers to build innovative, user-centered workflows on top of it. The value of CMS piloting these utilities would be to ensure that the data made available by payers and providers is complete, accurate, and usable, so that it can be used by CMS and others. Successful CMS reference implementations would prove to the industry that data in the APIs and MRFs are now usable, and can be leveraged to build useful consumer apps.

The greatest legacy CMS could leave for the health tech ecosystem is the establishment of a reliable, accurate, and complete flow of data from payers and providers into the broader industry. This infrastructure would enable innovation across countless workflows, beyond what any single CMS-developed app could achieve.

Data Access and Integration

PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

a. Examples may include the following:

The most valuable health data use cases are those that empower patients (and employers) to make better informed decisions, driving competition among providers and plans, and improving care access and affordability. These include:

- *In-network status – Accurate information on which providers participate in which plans enables patients to (a) select providers that accept their insurance and (b) choose health plans that include their preferred providers. When patients choose plans based on accurate provider participation data, plans are incentivized to contract with the providers that patients prefer. This alignment improves*

satisfaction and access, and reduces friction in care delivery. Payers could integrate data on their networks, competitive payer networks, and population health data to facilitate more sophisticated supply-demand alignment.

- *Cost estimation – Providing Medicare beneficiaries with access to out-of-pocket cost estimates empowers informed and value-conscious decision-making. While high-deductible plans are uncommon in Medicare Advantage (MA), many enrollees face cost-sharing responsibilities such as copays and coinsurance. Transparent cost information allows beneficiaries to anticipate expenses before receiving care, reducing surprise bills and encouraging the selection of lower-cost, high-quality providers. Over time, this can promote competition and efficiency among providers. Additionally, giving entities such as retiree health plan sponsors and care navigators access to negotiated rates can support smarter referrals that steer members toward more affordable, high-value care.*

- *Schedule availability – In terms of the provider selection use case, when patients can see actual appointment availability, they will prefer (and select) providers who can see them sooner. Shifting to the plan selection use case, this same appointment information, when aggregated, can help patients compare plans based on network availability, not just network size. Health plans would then be incentivized to build networks that offer not only breadth but timely access to care, fostering meaningful competition on network availability. Injecting availability as a measure of care access will be an important new dimension of evaluating and comparing providers and provider networks.*

When these datasets are accurate, standardized, and publicly accessible, they unlock a virtuous cycle of innovation and competition, prompting providers and health plans to respond more dynamically to patient needs and preferences.

b. What use cases are possible today?

In-network status – This use case is technically possible today, but limitations remain. While most MA plans have Provider Directory APIs, some of them exhibit issues that prevent developers from querying at scale. This includes lack of practitioner-plan relationships, lack of NPIs, incomplete data, and lack of key query parameters (e.g., unable to query by City, State, or zipcode). More specific requirements, monitoring of these APIs, and enforcement are important to have a complete picture of all MA networks to support provider search and plan comparison for beneficiaries.

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Cost estimation – Transparency in coverage (TiC) files are not yet required for Medicare Advantage plans, so CMS would need to require that MA plans publish these. Even when these files are available for commercial plans, the data is difficult to work with. Machine-readable files often include discrepant rates, outdated "zombie" rates, and redundant or excessively large datasets. There are few consumer-facing tools built on this data, largely because cleaning and interpreting the data requires access to historical claims and advanced processing to make the information usable.

c. What should be possible in the near future?

With stronger monitoring and enforcement, in-network status and cost estimation should become universally accessible and reliable. Developers have been documenting, organizing, and reporting issues for Provider Directory APIs, and payer and provider Price Transparency files since they became available. Regulators like CMS have ready access to issue reports submitted by developers, but only limited follow-up action has occurred. Provider directory APIs and price transparency files could be standardized, complete, and consistently published, resolving the most common issues observed. This would enable apps and services to deliver accurate information to patients. These improvements would then lay the groundwork for better care navigation and more effective competition among health plans and providers.

d. What would be very valuable but may be very hard to achieve?

- *Schedule availability – This is one of the most valuable yet elusive data elements. Today, schedule data is not available in a machine-readable or standardized format, nor is it mandated. It's only accessible when providers voluntarily share it with appointment aggregators or when bespoke agreements with payers (or federal agencies like VA) exist. Without comprehensive access to this data, patients face unnecessary delays and inefficiencies in finding care.*
- *Digital quality measures – Reliable, digital, and standardized quality data, tied to individual providers and care episodes, remains difficult to access and use. These data are essential for patients and plans to make informed decisions.*

The real opportunity lies in the integration of schedule availability, cost data, quality measures, and in-network status into a seamless consumer experience for Medicare beneficiaries. This won't happen until the underlying data are accurate, complete, and consistently available across all payers and providers.

When trusted data is available, it can lead to stakeholders making decisions that reward those industry participants contributing greatest value. Patients and employers select

plans that offer high-value providers (those with lower cost, higher quality, and availability). Plans compete to contract with providers who meet these criteria to attract members. Providers are incentivized to improve performance to remain competitive and in-network. Patients make informed decisions, reducing unnecessary costs and delays.

The result is a more responsive and patient-centered healthcare system where competition is based on performance that benefits the beneficiary, not opacity.

Providers, Digital Health Apps

PR-4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

To enable seamless patient experiences through third-party digital health applications, several critical improvements to standards and enforcement mechanisms are needed:

1. *Stronger Monitoring and Enforcement of Payer APIs - Payer-facing APIs (including the Provider Directory API, Patient Access API, and Prior Authorization API (once implemented)) must be regularly monitored for:*
    - *Availability (is the API up and responsive?)*
    - *Standards conformance (e.g., HL7 FHIR compliance)*
    - *Queryability - Is all data in the API queryable? (Querying a provider by Location. Querying providers who accept a particular plan.)*
        - *i. The most common issue that Defacto Health observes in payers' Provider Directory APIs is the absence of logical relationships between healthcare providers and insurance plans.*
        - *ii. Knowing that a provider is in-network for a particular plan is the #1 job that many beneficiaries use a payer's directory for.*
        - *iii. The absence of that logical relationship, and the inability to determine that a provider accepts a particular plan (not just in any network operated by a carrier) is unacceptable and a fundamental failure for APIs to accomplish the job-to-be-done.*
    - *Data quality (accuracy, completeness, freshness) - Too often, the data being made available via APIs is incomplete or inaccurate. Very basic checks of the data can be automated to check for conformity to real world information and usability relative to priority use cases.*
        - *i. Defacto Health monitors the relative accuracy of data coming from 140+ payer's Provider Directory APIs using a combination of*

algorithmic and phone-based audit methods. We observed that mean accuracy is 48% (aligned with documented Medicare Advantage monitoring methodology), with maximum accuracy over 70% and minimum observed accuracy less than 30%.

ii. *High quality data is possible and achievable. There are payers that we identify as 'positive outliers' that are out-performing other payers in terms of their directory accuracy. It is possible for regulators such as CMS to have this expectation of payers, to publish an explicit definition for accuracy, and to begin holding both payers and providers accountable for the accuracy of directory data.*

2. *Improved Oversight of Price Transparency Files - Both hospital and payer machine-readable files (MRFs) for price transparency should be:*
    - *Required for Medicare Advantage plan relationships*
    - *Checked for conformance to schema standards*
    - *Evaluated for usability (e.g., presence of duplicate or outdated rates, formatting issues, completeness). Currently, many files are unusable due to poor formatting, redundancies, and data inconsistencies. Stronger enforcement is needed when MRFs are missing or are unusable.*

3. *Mandate Appointment Availability APIs from Providers - Provider organizations should be required to expose appointment availability data via open APIs. This is essential for enabling third-party apps to offer patients the ability to:*
    - *View appointment slots in real time*
    - *Book and modify appointments directly through apps - These APIs should follow emerging FHIR scheduling standards (e.g., Argonaut scheduling), and EHR vendors should be required to support these endpoints. This would dramatically reduce friction in accessing timely care.*
    - *Know availability-adjusted network adequacy scores - Payers may be able to communicate more impactful access metrics to beneficiaries, letting them know the true availability of their networks. This could also be a metric considered by regulators for payer monitoring and submission.*

More on appointment scheduling and directories:
https://defacto.health/2024/11/23/payer-provider-directories-appointment-scheduling/

4. *Establish a Standard for Inbound Patient Intake (Clipboard API) - A standard "clipboard" or intake data ingestion API should be created to allow apps to send structured patient-generated data directly into the provider's EHR before a visit.*

EHR vendors should be required to support this capability, enabling a smoother pre-visit experience and reducing burden on patients and front office staff.

- *Demographics, Medical history, Insurance information*

Data Exchange

PR-5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

c. Provider Directory API.

Defacto Health serves as an intermediary between payers' Provider Directory APIs and the broader ecosystem that relies on provider and network data, including technology vendors, patient-facing platforms, provider organizations, health insurance payers, government agencies, and life sciences companies. We operate a structured process to ensure the availability, reliability, and usability of provider directory data:

1. *API Discovery - We identify payer Provider Directory APIs, and machine-readable files, and validate their endpoint availability and coverage.*

2. *Profiling - We analyze each API to understand its structure, supported query parameters, and any deviations from the required standards (e.g., FHIR, Da Vinci PDex). This allows us to tailor effective querying strategies.*

3. *Testing - We test each API to evaluate whether:*
   - *All expected provider data is present*
   - *Data is queryable using supported filters (e.g., location, specialty, network)*
   - *The API content is aligned with the payer's public-facing web directory or printed directory, ensuring consistency and completeness*

4. *Ongoing Querying and Monitoring - We perform regular, high-volume querying to ingest updates from each payer's Provider Directory API. This ensures that downstream users are working with the most up-to-date information.*

5. *Normalization - We standardize data across payers to a common schema, resolving differences in format, terminology, and structure.*

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6. *Data Delivery - Cleaned and normalized data is made available to our customers via structured outputs that power a wide range of healthcare use cases.*

While the majority of payers support only transactional API queries (e.g., search for providers in a particular zip code), we currently have access to bulk data from two payers. Bulk data access allows us to download network-wide directory data sets in one fell swoop. Wider availability of bulk APIs would improve efficiency and completeness.

Current Provider Directory APIs fall short in key areas that directly affect Medicare beneficiaries' ability to confidently find in-network care. Many of the APIs in use today are not performant or scalable enough to support real-time or bulk queries, limiting their utility in both health plan and third-party applications. Defacto observes and reports issues in payers' Provider Directory APIs and encourages the payers to resolve these functional issues. When these go unresolved, and workarounds are possible, Defacto implements non-standard querying methods to access and query the data. More explicit requiredness around data and query parameters would render these workarounds unnecessary and open up more payers' data to more developers.

Data quality remains a major issue: provider directories often include outdated or inaccurate information, such as providers no longer practicing at a listed location, non-functional phone numbers, or providers who do not accept the listed insurance plan. These data quality issues have multiple causes: providers submitting inaccurate data to payers, errors in logic within payers' systems, and lack of robust data governance across multiple 'source of truth' databases. For MA enrollees, this can lead to delays in care, higher out-of-network costs, or abandonment of needed services.

The situation is compounded by other structural limitations:

- *Ambiguity in plan and network naming, making it hard to distinguish between similarly named options. Limited use of Plan Benefit Package (PBP) IDs.*
- *Frequent absence of key logical relationships between entities (e.g., practitioner and plan, practitioner and organization, practitioner and location).*
- *Inconsistent use of NPIs within APIs to represent providers*

Opportunities for Improvement - To improve the experience for Medicare beneficiaries and others, the following actions should be considered:

- *Require essential logical relationships (e.g., practitioner and plan, practitioner and organization, practitioner and location) to be present within the APIs*
- *Require FHIR-based bulk query capabilities for better data availability at scale.*

- *Establish a universal provider directory accuracy standard, and publicly report accuracy rates by payer, with associated penalties and incentives.*
- *Require providers to publish their own up-to-date directory information in standardized, machine-readable formats (e.g., schema.org) on their websites.*
- *Ensure every provider and organization is represented with an NPI in APIs.*
- *Develop a universal network ID for consistent reference to plan networks.*
- *Mandate appointment search and booking APIs to allow patients to filter providers based on availability, building on existing work by VA and Argonaut.*

Video demonstrations:

1) *Provider Directory APIs that work: https://www.youtube.com/watch?v=KUwH-yWZi34*
2) *Provider Directory APIs that don't work: https://youtu.be/umeKb7jUod0?si=-QDYwSjkStBkrIqu*

How These Changes Will Help Medicare Beneficiaries

- *More accurate directories will reduce the risk of beneficiaries selecting out-of-network providers or encountering closed practices.*
- *Real-time appointment data will allow patients to identify providers who are both in-network and accepting new patients.*
- *Plan comparisons will improve as beneficiaries can evaluate the actual breadth and availability of each provider network.*
- *A more functional provider search experience will reduce frustration and improve access to timely, affordable care for Medicare beneficiaries.*

These improvements can deliver a more transparent, efficient, and trustworthy healthcare experience for all Medicare beneficiaries.

Payers

PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

A nationwide provider directory with FHIR endpoints and digital identity credentials would offer transformative value to payers in the following ways:

1. Improved Access to Clinical Data - With reliable FHIR endpoints associated with verified digital identities, payers could more efficiently retrieve clinical data from

providers for HEDIS quality measurement, risk adjustment, and value-based care initiatives. This reduces reliance on manual chart retrieval or proprietary data exchange channels, improving timeliness, completeness, and lowering administrative costs.

2. Streamlined Credentialing and Reduced Redundancy - If provider credentials, such as licensure, board certifications, and education degrees, were digitally signed by primary sources (e.g., state boards, ABMS, medical schools), and accepted by accreditation bodies such as NCQA and The Joint Commission, payers could:

- *Rely on digitally verified credentials for network participation*
- *Reduce redundant verification across Medicare FFS, Medicaid, Medicare Advantage, Medicaid MCOs, and ACA plans*
- *Significantly lower credentialing costs and reduce provider abrasion*

3. Centralized Source of Accurate Directory Data - By collecting foundational directory data (e.g., provider name, NPI, practice location, phone number, organizational affiliation) in a single authoritative source:

- *Accuracy can be centrally monitored and enforced*
- *Duplicative data collection efforts by each payer are avoided*
- *Errors, outdated information, and administrative inefficiencies are minimized*

Technical Standards and Certification

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS can catalyze greater use of standards-based APIs by ensuring that the infrastructure it has already mandated is truly available, usable, and trustworthy:

1. Ensure Mandated APIs Are Published and Discoverable

- *Enforce that all required APIs are live, including:*
  - *Patient Access APIs (from payers and providers)*
  - *Provider Directory APIs*
  - *Prior Authorization APIs*
  - *Provider Access APIs*
- *Conduct regular audits or monitoring to ensure these APIs are not only technically available but also discoverable and functioning.*

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2. Ensure Conformance to Standards

- *APIs must be fully conformant to FHIR and associated IGs, without requiring special adaptation or organization-specific query logic.*
- *Use of implementation guides (IGs) should result in predictable, reusable patterns that developers can rely on without customization.*

3. Ensure Data Completeness and Accuracy

- *APIs should expose complete and current data*
- *CMS should monitor for data quality and consistency between API responses and payer/provider public-facing sources (e.g., web directories, EHRs).*

4. Incentivize Trust Through Certification or Accreditation

- *Establish a certification program that gives APIs a "seal of approval" for availability, conformance, data quality, and performance.*
- *Publicly list functioning APIs to help app developers and other ecosystem actors prioritize trustworthy APIs over incomplete or flawed APIs.*

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

A nationwide directory of FHIR endpoints would meaningfully improve access to health information across the healthcare ecosystem. Patients would benefit by more easily using their Patient Access APIs to retrieve data, especially when changing health plans or providers. Providers would be better able to access patient data from payers via Provider Access APIs, supporting continuity of care, value-based care delivery, and quality measurement. Payers and providers could also use these endpoints for business-to-business use cases, provided the APIs are available, functional, and conformant to standards. A centralized directory would streamline the discovery of these endpoints and help ensure reliable access to information.

Today, this type of infrastructure does not exist comprehensively and publicly. The effort involved in discovering, verifying, and maintaining FHIR endpoints is substantial, and multiple private entities have responded by developing proprietary, internal directories for their own use. While this has served a short-term market need, it fragments the ecosystem and limits the broader potential of interoperable infrastructure.

Rather than building and maintaining the directory entirely within government, a more feasible approach would be for CMS or ASTP to establish the core infrastructure and designate nonprofit stewards to manage key portions of the directory. These could include the American Medical Association for physicians, CAQH for payer networks, and the Federation of State Medical Boards for licensure and certification. Because these entities often serve as the primary sources of the underlying data, they are well-positioned to both contribute to and benefit from a shared national directory. A multi-stakeholder governance board could provide oversight and ensure accountability.

Some level of cost-sharing would be appropriate. While government support may cover the core infrastructure, contributors and users of the directory could bear a portion of the costs. These fees would be offset for participants by reductions in administrative burden. In addition, participation in the national directory could entitle users to certain regulatory relief. For instance, agencies could exercise enforcement discretion around compliance with provider directory accuracy for entities that use the national directory in good faith. For providers whose EHR instances (and associated Patient Access APIs) are certified and published on a national directory, and sign TEFCA-like agreements, they could be deemed compliant with ASTP information blocking requirements.

A final note: CMS should consider collaborating with the US Department of Veterans Affairs on national directory efforts. They released an RFI recently on VA Community Care Provider Directory. According to the RFI, they are seeking to establish a 'source of truth' database within their own organization for provider directory data, setting ambitious goals on unprecedented levels of directory accuracy (> 90%), and seeking dynamic appointment booking capabilities with community providers (that would require exposing appointment scheduling endpoints with their EHRs). There are significant overlapping goals between VA and CMS, and it would be an appropriate opportunity to collaborate to accelerate progress and to minimize redundant efforts in government.

More about potential VA and CMS collaboration:
https://defacto.health/2025/05/22/va-and-cms-fixing-provider-directories-together/

TD-19. Regarding price transparency implementation:

a. What are current shortcomings in content, format, delivery, and timeliness?

Medicare Advantage plans are not currently required to share TiC machine-readable files, so answers to this question are relative to currently available MRFs.

Despite the availability of machine-readable files under current price transparency regulations, shortcomings remain in enforcement, data quality, and usability. Many

payers and providers are not fully compliant, and even among those who are, the data often suffer from accuracy and completeness issues such as missing NPI lists, unexplained rate conflicts, and inclusion of "zombie rates" for services not actually offered by a provider. Rates are typically listed at the procedure code level, making it difficult for patients to understand the total cost of an episode of care. In addition, the lack of standard network identifiers makes it hard to align pricing data with a patient's specific plan, and there is no reliable way to connect pricing to coverage rules. The absence of intuitive service bundles and practical Good Faith Estimate workflows further limits the usefulness of price data for patients and referring providers.

b. Which workflows would benefit most from functional price transparency?

Functional price transparency would enhance several workflows, particularly those related to provider selection. Patients, especially those with cost-sharing requirements, could more confidently compare costs across providers. Referring providers could better advise patients on cost-effective care pathways. Additionally, providers could begin to respond to requests for cost estimates that reflect the full scope of care, including services outside their own organizations, if data exchange standards were in place.

c. What improvements would be most valuable for patients, providers, or payers, including CMS?

For all stakeholders, greater consistency, accountability, and clarity in price data would yield the most value. Patients would benefit from clear, episode-based price estimates tied to their coverage and network, supported by real-time APIs for Good Faith Estimates. Providers would gain the ability to create more accurate and complete GFEs if payer-provider data exchange standards were finalized and adopted.

Payers and CMS would benefit from more accurate pricing data to support oversight, program integrity, and market functioning. Enhancements such as requiring payers to explain conflicting rates, provide utilization context around negotiated rates, and adopt standard network identifiers would improve the usability of the data. A system for public data quality evaluation of all published price transparency data and issue reporting would reinforce accountability on payers and providers.

d. What would further motivate solution development?

A combination of regulatory pressure, practical implementation support, and meaningful incentives would help accelerate development. Stronger monitoring and enforcement for non-compliance would create a level playing field and reward those who invest in doing

it right. Standards development and real-world pilots, particularly around GFEs and Advanced EOBs, would give the industry clarity and confidence.